

Pharmacode position may change as per Supplier's m/c requirement &additional  
small pharma code may appear on the front / back panel



**AUROCEF**  
Cefalexin Capsules 250 mg and 500 mg

**Description and composition:**

Aurocef 500mg:

Dark blue opaque/light blue opaque size “0” hard gelatin capsule filled with off white granular powder and imprinted with 'A76' on dark blue opaque Cap and '500 mg' on light blue opaque body with black ink.

Each capsule contains: Cephalexin BP equivalent to Cephalexin anhydrous 500 mg Aurocef 250mg:

Dark blue opaque/white size '2' hard gelatin capsule filled with off white granular powder and imprinted with 'A75' on dark blue opaque cap and '250mg' on white body with black ink.

Each capsule contains: Cephalexin BP equivalent to Cephalexin anhydrous 250 mg

**Pharmacodynamics:**

*In vitro* tests demonstrate that cephalosporins are bactericidal because of their inhibition of cell-wall synthesis.

Cephalexin is active against the following organisms *in vitro*:

Beta-haemolytic streptococci

Staphylococci, including coagulase-positive, coagu-lase-negative, and penicillinase-producing strains

*Streptococcus pneumoniae*

*Escherichia coli*

*Proteus mirabilis*

*Klebsiella species*

*Haemophilus influenzae*

*Branhamella catarrhalis*

Most strains of enterococci (*Streptococcus faecalis*) and a few strains of staphylococci are resistant to cefalexin. It is not active against most strains of *Enterobacter* species, *Morganella morganii*, and *Pr. vulgaris*. It has no activity against *Pseudomonas* or *Herellea species* or *Acinetobacter calcoaceticus*. Penicillin-resistant *Streptococcus pneumoniae* is usually cross resistant to beta-lactam antibiotics. When tested by in vitro methods, staphylococci exhibit cross-resistance between cefalexin and methicillin-type antibiotics.

**Pharmacokinetics:**

Cefalexin is acid stable and may be given without regard to meals. It is rapidly absorbed after oral administration. Following doses of 250mg, 500mg, and 1g, average peak serum levels of approximately 9, 18, and 32mg/l, respectively, were obtained at 1 hour.

Measurable levels were present 6 hours after administration. Cefalexin is excreted in the urine by glomerular filtration and tubular secretion. Studies showed that over 90% of the drug was excreted unchanged in the urine within 8 hours. During this period, peak urine concentrations following the 250mg, 500mg, and 1g doses were approximately 1,000, 2,200, and 5,000mg/l, respectively.

Cefalexin is almost completely absorbed from the gastro-intestinal tract, and 75-100% is rapidly excreted in active form in the urine. Absorption is slightly reduced if the drug is administered with food. The half-life is approximately 60 minutes in patients with normal renal function. Haemodialysis and peritoneal dialysis will remove cefalexin from the blood. Peak blood levels are achieved one hour after administration, and therapeutic levels are main-tained for 6-8 hours. Approximately 80% of the active drug is excreted in the urine within 6 hours. No accumulation is seen with dosages above the therapeutic maximum of 4g/day. The half-life may be increased in neonates due to their renal immaturity, but there is no accumulation when given at up to 50mg/kg/day.

**Indication**

Cephalexin is a semi-synthetic cephalosporin antibiotic for oral administration.

Cephalexin is indicated in the treatment of the following infections due to susceptible microor-ganisms:

Respiratory tract infections

Otitis media

Skin and soft tissue infections

Bone and joint infections

Genito-urinary tract infections, including acute prostatitis

Dental infections

**Recommended Dose**

Adults: Many infections in adults will respond to oral dosage of 1 gram to 2 grams per day in divided doses; however, for most infections, the following simple scheme will be found satisfactory:

Adult and children over 12 years: 500mg three times daily.

To aid compliance, especially in ambulatory patients, the daily dosage may be given in two equal doses, e.g. 1 g twice daily in adults with urinary tract infections.

For severe or deep seated infections, especially when less sensitive organisms are involved, the dosage should be increased to 1 g three times daily or 1.5 g four times daily.

Children: Normal recommended dose for children: 25-60mg/kg/day. For chronic, severe or deep seated infections, this should be increased to 100 mg/kg/day (maximum 4g/day). 0 to 1 year : 25 to 60 mg/kg/day

1 to 2 years : 65.5 to 125 mg four times daily or 125 to 250 mg twice daily.

3 to 6 years: 125 to 250 mg four times daily or 250 to 500 mg twice daily.

7 to 12 years: 250 to 500mg four times daily or 500 mg to 1 g twice daily

**Route of administration**

Aurocef is administered orally.

**Contraindication**

Cephalexin is contra-indicated in patients with known allergy to the cephalosporin group of antibiotics.

**Warning and precaution**

Before instituting therapy with Cephalexin, every effort should be made to determine whether the patient has had previous hypersensitivity reactions to the cephalosporins, penicillins, or other drugs. Cephalexin should be given cautiously to penicillin-sensitive patients. There is some clinical and laboratory evidence of partial cross-allergenicity of the penicillins and cephalosporins. Patients have had severe reactions (including anaphylaxis) to both drugs.

Pseudomembranous colitis has been reported with virtually all broad-spectrum antibiotics, including macrolides, semi-synthetic penicillins, and cephalo-sporins. It is important, therefore, to consider its diagnosis in patients who develop diarrhoea in association with the use of antibiotics. Such colitis may range in severity from mild to life threatening. Mild cases of pseudomembranous colitis usually respond to drug discontinuance alone. In moderate to severe cases, appropriate measures should be taken.

If an allergic reaction to cefalexin occurs, the drug should be discontinued and the patient treated with the appropriate agents.

Prolonged use of cefalexin may result in the overgrowth of non-susceptible organisms. Careful observation of the patient is essential. If super infection occurs during therapy, appropriate measures should be taken.

Cephalexin should be administered with caution in the presence of markedly impaired renal function. Careful clinical and laboratory studies should be made because safe dosage may be lower than that usually recommended.

Positive direct Coombs' tests have been reported during treatment with the cephalosporin antibiotics. In haematological studies, or in transfusion cross-matching procedures when antiglobulin tests are performed on the minor side, or in Coombs' testing of newborns whose mothers have received cephalosporin antibiotics before parturition, it should be recognized that a positive Coombs' test may be due to the drug.

A false positive reaction for glucose in the urine may occur with Benedict's or Fehling's solutions, or with copper sulphate test tablets.

**Interaction with other medicaments**

As with other beta-lactam drugs, renal excretion of Cephalexin is inhibited by probenecid. In a single study of 12 healthy subjects given single 500mg doses of cefalexin and metformin, plasma metformin C<sub>max</sub> and AUC increased by an average of 34% and 24%, respectively, and metformin renal clearance decreased by an average of 14%. No side effects were reported in the 12 healthy subjects in this study. No information is available about the interaction of Cephalexin and metformin following multiple dose administration. The clinical significance of this study is unclear, particularly as no cases of “lactic acidosis” have been reported in association with concomitant metformin and cefalexin treatment.

**Pregnancy and lactation**

Usage in pregnancy: Although laboratory and clinical studies have shown no evidence of teratogenicity, caution should be exercised when prescribing for the pregnant patient.

Usage in nursing mothers: The excretion of cefalexin in human breast milk increased up to 4 hours following a 500mg dose. The drug reached a maximum level of 4 micrograms/ml, then decreased gradually and had disappeared 8 hours after administration. Caution should be exercised when cefalexin is administered to a nursing woman.

**Side effects**

Gastro-intestinal: Symptoms of pseudomem-branous colitis may appear either during or after antibiotic treatment. Nausea and vomiting have been reported rarely.

A/s : 210 x 300 mm ■ Black

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| <div><br/><b>AUROBINDO</b><br/>Packaging Development</div> |            | Product Name       | Component                                                                                                | Item Code       | Date & Time            |
|                                                                                                                                               |            | Aurocef Caps       | Leaflet                                                                                                  | P1513815        | 16.04.2020 & 3:10 PM   |
|                                                                                                                                               |            | Customer / Country | Version No.                                                                                              | Reason Of Issue | Reviewed / Approved by |
| Malaysia                                                                                                                                      | 02         | Revision           |                                                                                                          |                 |                        |
| Team Leader                                                                                                                                   | Lova Raju  | Dimensions         | No. of Colours : 01                                                                                      |                 |                        |
| Initiator                                                                                                                                     | Bharath    | 210 x 300 mm       | <div><br/>13815</div> |                 |                        |
| Artist:<br>                                                | Pharmacode | 13815              |                                                                                                          |                 |                        |
|                                                                                                                                               |            |                    |                                                                                                          |                 |                        |
| Additional Information :      Supersede Item Code: P1503341      WOS-17-000121                                                                |            |                    |                                                                                                          |                 | Sign / Date            |

The most frequent side effect has been diarrhoea. It was very rarely severe enough to warrant cessation of therapy. Dyspepsia and abdominal pain have also occurred. As with some penicillins and some other cephalosporins, transient hepatitis and cholestatic jaundice have been reported rarely.

Hypersensitivity: Allergic reactions have been observed in the form of rash, urticaria, angioedema, and, rarely, erythema multiforme, Stevens-Johnson syndrome, and toxic epidermal necrolysis. These reactions usually subsided upon discontinuation of the drug, although in some cases supportive therapy may be necessary. Anaphylaxis has also been reported.

Haemic and lymphatic system: Eosinophilia, neutropenia, thrombocytopenia, and haemolytic anaemia have been reported.

Other: These have included genital and anal pruritus, genital candidiasis, vaginitis and vaginal discharge, dizziness, fatigue, headache, agitation, confusion, hallucinations, arthralgia, arthritis, and joint disorder. Reversible interstitial nephritis has been reported rarely. Slight elevations in AST and ALT have been reported.

**Symptoms and treatment of overdose**

Symptoms of oral overdose may include nausea, vomiting, epigastric distress, diarrhoea, and haematuria.

In the event of severe overdosage, general supportive care is recommended, including close clinical and laboratory monitoring of haematological, renal, and hepatic functions, and coagulation status until the patient is stable. Forced diuresis, peritoneal dialysis, haemodialysis, or charcoal haemoper-fusion have not been established as beneficial for an overdose of cefalexin. It would be extremely unlikely that one of these procedures would be indicated.

Unless 5 to 10 times the normal total daily dose has been ingested, gastro-intestinal decontamination should not be necessary.

There have been reports of haematuria, without impairment of renal function, in children accidentally ingesting more than 3.5g of cefalexin in a day. Treatment has been supportive (fluids) and no sequelae have been reported.

**Storage condition**

Store in a dry place at a temperature not exceeding 30°C, protect from light.

**Shelf life**

24 months from the date of manufacture

**Presentation**

Aurocef 250 and 500mg:

- 1)10 blisters of 10 capsules are packed in a printed carton along with a package insert.
- 2) 100 blisters of 10 capsules are packed in an E-flute Box along with a package insert.

**Manufactured By**

  
**AUROBINDO**  
Aurobindo Pharma Limited (Unit – VI)  
Survey: No. 329/39 & 329/47 Chitkul  
village, Patancheru Mandal, Sanga  
Reddy District,  
Telangana State  
India

**PRODUCT REGISTRTION HOLDER  
(PRH):**

UNIMED SDN BHD 53, Jalan Tembaga  
SD 5/2B, Bandar Sri Damansara, 52200  
Kuala Lumpur, Malaysia

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